

1. Administrative & Study Identification

Full Study Title: A Single-arm Multi-center Study of Ruxolitinib in Chinese Participants With Corticosteroid-refractory Chronic Graft Versus Host Disease After Allogeneic Stem Cell Transplantation **Short Title/Acronym:** SR-cGvHD Ruxolitinib Study **Protocol Number:** CINC424D2413 **NCT Number:** NCT06824103 **Sponsor/Company Name:** Novartis Pharmaceuticals **Investigational Product (Drug Name):** Ruxolitinib (INC424) **Phase of Development:** Phase IV (PHASE4) **Trial Status:** RECRUITING (Currently recruiting participants) **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of ruxolitinib, measured by Overall Response Rate (ORR), in Chinese adult and pediatric participants (aged ≥ 12 years) with corticosteroid-refractory chronic graft vs. host disease (SR-cGvHD). **Secondary Objectives:** To evaluate the Duration of Response (DOR), Failure-free Survival (FFS), Best Overall Response (BOR), Overall Survival (OS), Non-Relapse Mortality (NRM), malignancy relapse/recurrence rate, and the reduction in systemic corticosteroid use.

2.2 Background & Rationale To address the medical necessity of effective treatments for patients who have developed corticosteroid-refractory or corticosteroid-dependent chronic graft-versus-host disease (cGvHD) following allogeneic stem cell transplantation (alloSCT). Patients failing standard corticosteroid therapy have limited clinical options and require targeted interventions like the JAK1/2 inhibitor ruxolitinib to alleviate symptoms, achieve disease response, and reduce long-term steroid dependence.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A **Criteria Requirement:** Trial must be in PHASE4 (Phase_ID: PHASE4, Source Level: 5)

3.2 Planned Enrollment & Duration Targeted Enrollment (\$N\$): 50 participants **Number of Sites:** Multi-center across China **Estimated Study Start:** Late 2025 **Study Duration & Periods:** Total duration on study for an individual participant is up to 164 weeks (approximately 3 years). The study consists of the following

periods, with each cycle comprised of 4 weeks (28 days): * Screening Period (Day -28 to Day -1) * Treatment period (Day 1 to Cycle 39/EOT) * Safety follow-up (Last dose +30 days) * Long-term survival follow-up period (EOT to 156 weeks on study)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Signed informed consent must be obtained prior to participation in the study. 2. Male or female Chinese participants aged ≥ 12 years at the time of informed consent. 3. Have undergone alloSCT from any donor source (matched unrelated donor, sibling, haplo-identical) using bone marrow, peripheral blood stem cells, or cord blood. Recipients of non-myeloablative, myeloablative, and reduced intensity conditioning are eligible. 4. Clinically diagnosed cGvHD staging of moderate to severe according to NIH Consensus Criteria (Jagasia et al 2015) prior to Cycle 1 Day 1. * Moderate cGvHD: at least one organ (not lung) with a score of 2, 3 or more organs involved with a score of 1 in each organ, or lung score of 1. * Severe cGvHD: at least 1 organ with a score of 3, or lung score of 2 or 3. 5. Currently receiving systemic corticosteroids for the treatment of cGvHD for a duration of < 12 months prior to Cycle 1 Day 1, with corticosteroid-refractory cGvHD (e.g., lack of response or disease progression after administration of ≥ 1 mg/kg/day prednisone for at least 1 week, disease persistence at > 0.5 mg/kg/day for at least 4 weeks, or increase to > 0.25 mg/kg/day after two unsuccessful attempts to taper). 6. Evident myeloid and platelet engraftment: Absolute neutrophil count (ANC) $> 1,000/\text{mm}^3$ AND Platelet count $\geq 25,000/\text{mm}^3$. (Note: Transfusion to reach a minimum platelet count for inclusion is not allowed during screening and at baseline). 7. Participant has Eastern Cooperative Oncology Group (ECOG) performance status of 0-2. 8. Able to swallow tablets.

4.2 Exclusion Criteria 1. Failed prior alloSCT within the past 6 months from Cycle 1 Day 1. 2. Participants with relapsed primary malignancy, or who have been treated for relapse after the alloSCT was performed. 3. Participants who have received two or more systemic treatments for cGvHD in addition to corticosteroids $\pm$ CNI. 4. Prior treatment with ROCK2 inhibitors for cGvHD. 5. Treated with prior JAK inhibitors for aGvHD (unless complete or partial response was achieved and patient has been off JAK inhibitor treatment for at least 8 weeks prior to Cycle 1 Day 1). 6. Participants that transition from active aGvHD to cGvHD without tapering off corticosteroids $\pm$ CNI and any systemic treatment. 7. SR-cGvHD occurring after a non-scheduled donor lymphocyte infusion (DLI) administered for pre-emptive treatment of malignancy recurrence. (Participants who received a scheduled DLI as part of transplant and not for malignancy relapse are eligible).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ruxolitinib 10 mg BID (twice a day), given as two 5 mg tablets. **Route of Administration:** Oral. **Trade Names:** Jakafi, Opzelura

5.2 Concomitant & Prohibited Therapy Permitted: Growth factor supplementation and transfusion support; continued baseline corticosteroids (tapering is encouraged); participants receiving up to 30 mg by mouth once a day of hydrocortisone (i.e., physiologic replacement dose) of corticosteroids are allowed. **Prohibited:** More than two prior systemic treatments for cGvHD; ROCK2 inhibitors; transfusion specifically to reach minimum baseline platelet inclusion criteria.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Day -28 to -1 | Informed Consent, Engraftment Labs (ANC, Platelets), Baseline NIH Scoring | | **Baseline** | Cycle 1 Day 1 | First Dose, Safety Audit | | **Interim** | Day 1 to Cycle 39/EOT | Cycle Visits (every 28 days): Efficacy Assessment (NIH Criteria), Steroid Dose Adjustments, Safety Labs | | **Safety Follow-up** | Last Dose +30 days | Post-treatment safety assessments | | **End of Study** | EOT to 156 weeks | Final Vital Signs, Exit Interview, Long-term Survival Follow-up (Total duration up to Wk 164) |

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes Overall Response Rate (ORR): ORR is defined as the percentage of participants demonstrating a complete response (CR) or partial response (PR) without the requirement of additional systemic therapies for an earlier progression, mixed response or non-response, according to National Institute of Health (NIH) Consensus Criteria.

7.2 Secondary Outcomes 1. Failure-free Survival (FFS) 2. Best Overall Response (BOR) 3. ORR at end of cycle 3 4. Duration of Response (DOR) 5. Overall Survival (OS) 6. Non-Relapse Mortality (NRM) 7. Reduction in systemic corticosteroid use (\$\ge 50\%\$ reduction from baseline)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of hematologic parameters (e.g., cytopenias such as low platelets, anemia, and neutropenia) and clinical status. **Serious Adverse Events (SAEs):** Standard 24-hour reporting for life-threatening events, opportunistic infections, or malignant relapses. **Data**

Monitoring Committee (DMC): Internal safety monitoring managed by Novartis.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy; Safety Set for all patients receiving at least one dose of ruxolitinib. **Sample Size Justification:** $N=50$ provides sufficient power to measure the Overall Response Rate in this specific, targeted cohort of Chinese patients with SR-cGvHD. **Handling of Missing Data:** Missing efficacy data or data from patients requiring the addition of new systemic therapies for cGvHD prior to evaluation milestones will generally be imputed as non-responders for the ORR endpoint.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine clinical monitoring for source data verification and adherence to NIH Consensus Criteria scoring. **Audit Trail:** Validated eCRF completion, data clarification processes, and strict accountability logs for the investigational product. **EMA Reference URL:** [Opzelura EPAR Product Information](#)

11. Study Identifiers & Governance

Primary ID: CINC424D2413 **Registry Number:** NCT06824103 **PostingID:** 486827818258007381 **Sponsor/Lead Organization:** Novartis **Study Title:** A Single-arm Multi-center Study of Ruxolitinib in Chinese Participants With Corticosteroid-refractory Chronic Graft Versus Host Disease After Allogeneic Stem Cell Transplantation **Official Regulatory Title:** Study of Efficacy and Safety of Ruxolitinib in Chinese Participants With Corticosteroid-refractory Chronic Graft vs. Host Disease

12. Clinical Framework (GvHD Specific)

Disease Areas: Graft vs. Host Disease, Chronic Graft vs. Host Disease, Corticosteroid-refractory Chronic Graft vs. Host Disease **Primary Condition (Name):** Graft vs. Host Disease (Preferred Name: Graft-vs-Host Disease; Preferred UMLS Name: Corticosteroid-Refractory Chronic Graft Versus Host Disease) **Semantic Type:** Disease or Syndrome **Definition:** The clinical entity characterized by anorexia, diarrhea, loss of hair, leukopenia, thrombocytopenia, growth retardation, and eventual death brought about by the GRAFT VS HOST REACTION. **Diagnosis Threshold:** Moderate to severe cGvHD according to NIH Consensus Criteria

Medical Methodology: Targeted JAK1/2 Inhibition (Ruxolitinib) following alloSCT **Clinical Taxonomy Codes:** * **MeSH Code:** D006086 * **SNOMED ID:** 234646005 * **ATC Code:** D11AH09

13. Study Procedures & Methodology

Management Pathway: Oral administration and systemic management of cGvHD **Education Protocol (HCP):** Standardization of NIH Consensus Criteria evaluations per site **Education Protocol (Patient):** Adherence to oral therapy regimen and regular reporting of GvHD symptoms **Quality Audit Mechanism:** Investigator assessment verification and survival tracking

14. Observation & Follow-Up Schedule

Total Duration: Up to 164 weeks **Onsite Visit Cadence:** Every 28 days (per Cycle) for active treatment up to Cycle 39/EOT **Remote Monitoring Frequency:** Standard safety check-ins as defined by local site protocol **Checklist Review Interval:** Every Cycle (28 Days)

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				